

7. STUDY CONDUCT

7.1. Schedule of Procedures

A schedule of events for the main study is provided in [Table 7](#). A schedule of events for patients participating in the open-label FE sub-study is provided in [Table 8](#). A schedule of events for the extended visit cycle for patients in the main study is provided in [Table 9](#).

Table 7 Schedule of Events – Main Study

Cycle ¹	Screening	1				2	Subsequent Cycles ²	Study Treatment Discontinuation (visit within 7 days of last dose)	Post Treatment Assessments
Day	-28 to -1	1 ³	8	15	21	1	Cycle n, Day 1		
Informed Consent ⁴	X								
Demographics	X								
Medical, surgical, cancer, medication history	X								
Record local gBRCA mutation result, if done ⁵	X								
Blood sample for centralized gBRCA mutation testing	X ⁶								
Blood sample for DNA repair typing		X							
Pregnancy test	X ⁷						X ⁷		

Abbreviations: C = Cycle; CA = cancer antigen; CBC = complete blood cell count; CT = computed tomography; D = Day; ECG = electrocardiogram; ECOG = Eastern Cooperative Oncology Group; eCRF = electronic case report form; EQ-5D-5L = European Quality of Life Scale, 5-Dimensions; FOSI = Functional Assessment of Cancer Therapy – Ovarian Symptom Index; gBRCA = germline BRCA; gBRCA^{mut} = germline BRCA mutation; ICF = informed consent form; MRI = magnetic resonance imaging; PET = positron emission tomography; PK = pharmacokinetic(s); PRO = patient reported outcome; RECIST = Response Evaluation Criteria in Solid Tumors; SAE = serious adverse event.

¹ Treatment cycles are 28 days long, visits on Day 1 of each cycle unless otherwise specified.

² Visits continue every 4 weeks until study treatment discontinuation.

³ C1/D1 is day of first dose.

⁴ A separate ICF may be signed prior to the screening period for Myriad Integrated BRACAnalysis[®] testing in order to facilitate early testing. Depending on local site requirements, patients may sign a single study ICF prior to the screening period to facilitate early gBRCA^{mut} testing only. All other study tests and procedures must be done in the screening window (Day -28 to Day -1).

⁵ Local gBRCA^{mut} results that had been performed previously should be entered into eCRF at screening.

⁶ Centralized gBRCA^{mut} testing of DNA by Integrated BRACAnalysis[®] at Myriad must be completed prior to randomization. The sample may be submitted prior to the screening period for patients who are identified for a Screening Visit to occur. DNA from this sample will be stored and may be used at a later time for biomarker testing including potential to bridging to candidate companion diagnostic assays.

⁷ Negative serum pregnancy test required within 72 hours from first dose of study treatment for females of childbearing potential; repeated every 3 months for duration of study (ie, Cycle 4, Cycle 7, etc).

Cycle ¹	Screening	1				2	Subsequent Cycles ²	Study Treatment Discontinuation (visit within 7 days of last dose)	Post Treatment Assessments
Day	-28 to -1	1 ³	8	15	21	1	Cycle n, Day 1		
Randomization	X ⁸								
Physical examination	X	X		X		X	X	X	
Vital signs, height, ⁹ weight	X	X		X		X	X	X	
ECOG performance status	X	X				X	X	X	
Adverse event monitoring	X	X		X		X	X	X ¹⁰	
Concomitant medications	X	X		X		X	X	X	
Coagulation/serum chemistry	X	X ¹¹		X		X	X	X	
CBC ¹²	X	X	X	X	X	X	X	X	
Serum CA-125 ^{13,14}	X	X				X	X	X	
Urinalysis ¹⁵	X								
12-lead ECG ¹⁶	X	X				X		X	

⁸ Randomization must occur within 72 hours prior to first dose.

⁹ Height obtained at screening only.

¹⁰ SAEs recorded up to 30 days after study treatment discontinuation.

¹¹ If Screening laboratory testing (serum chemistry, CBC, coagulation) performed within 72 hours of Day 1, repeat testing not required.

¹² If dose interruption or modification is required at any point on study because of hematologic toxicity, weekly blood draws for CBC will be monitored until the adverse event resolves, and to ensure safety of the new dose, weekly blood draws for CBC will be also be required for an additional 4 weeks after the adverse event has been resolved to the specified levels, after which monitoring every 4 weeks may resume.

¹³ CA-125 levels must be normal at screening or > 90% decrease as compared with baseline prior to last platinum-based chemotherapy course. Abnormal CA-125 levels during study do not represent disease progression; however, they may prompt imaging if clinically indicated. CA-125 sample within 72 hours of dose on Cycle 1/Day 1.

¹⁴ For determination of CA-125 progression, at least 2 CA-125 values at least 1 week apart are required for confirmation ([Section 6.1](#)).

¹⁵ Urinalysis parameters must include: specific gravity, leukocyte esterase, nitrite, blood, protein, glucose, ketones, urobilinogen, and bilirubin.

¹⁶ Patients will have a 12-lead ECG at Screening, Baseline (predose, within 30 minutes), 2 hours postdose on Day 1(within 30 minutes), Cycle 2/Day 1, (predose, 2 hours postdose, within 30 minutes) and upon study treatment discontinuation. Note that the ECG is to be completed prior to the PK blood draw.

Cycle ¹	Screening	1				2	Subsequent Cycles ²	Study Treatment Discontinuation (visit within 7 days of last dose)	Post Treatment Assessments
Day	-28 to -1	1 ³	8	15	21	1	Cycle n, Day 1		
Triplicate ECGs (<u>selected sites only</u>)		X ¹⁷							
Tumor sample ¹⁸	X								
Blood sample for PK		X ^{19,20}				X	X ²¹		
Tumor Assessment (RECIST) ²²	X						X	X	X
Chest CT/MRI ²³	X								
FOSI, EQ-5D-5L, neuropathy questionnaire ²⁴	X						X	X	X

¹⁷ At selected sites, a subset of patients (approximately 24) will undergo triplicate ECG testing on Day 1 only at baseline (predose) and 1, 1.5, 2, 3, 4, 6 and 8 hours postdose. Triplicate ECGs should be performed between 2-5 minutes apart and should be performed prior to blood draws for PK.

¹⁸ Formalin fixed, paraffin-embedded tumor sample (primary or metastatic site) consisting of 100 micron thickness of sections (≥ 80 micron minimum) or unsectioned paraffin block.

¹⁹ Blood samples for PK collected on Cycle 1/Day 1 and Cycle 2/Day 1 collected predose (within 30 minutes) and 2 hours postdose (within 30 $\pm$ 15 minutes Note: The exact time of the PK blood draw will be recorded and ECG monitoring is to be completed prior to the PK blood draw.

²⁰ At selected sites, a subset of patients (approximately 12) undergoing intensive PK on Day1 will have samples collected at baseline (within 30 minutes predose) and 1, 1.5, 2, 3, 4, 6 and 8 hours (± 2 minutes at all time points) postdose. Note: The exact time of the PK blood draw will be recorded and the ECG monitoring is to be completed prior to the PK blood draw.

²¹ Additional blood samples for PK on Cycle 4/Day 1 and Cycle 8/Day 1 will be collected at predose (trough) within 30 minutes only.

²² RECIST tumor assessment via CT or MRI scan of abdomen/pelvis and clinically indicated areas required at baseline, then after every 2 cycles (ie, 8 weeks with a window of ± 7 days from date of visit) through Cycle 14 (56 weeks), then after every 6 cycles (24 weeks with a window of ± 7 days) until study treatment discontinuation; at this point, a final follow-up set of imaging is required. PET/CT may be used according to RECIST guidelines. Cycle timing will not be delayed for treatment interruptions, and tumor assessment should occur according to this schedule regardless of whether study treatment is interrupted. If a patient discontinues treatment for clinical progression and does not meet the criteria specified in the protocol, scans and CA-125 testing should continue at the specified intervals until progression is confirmed or until the start of subsequent anticancer treatment.

²³ Chest CT/MRI if not done as part of RECIST tumor assessment at Screening. If the chest CT/MRI is clear at screening, repeat chest imaging is not required in the absence of lesions to be followed or in the absence of clinical indication requiring follow-up.

²⁴ PROs (FOSI, EQ-5D-5L, neuropathy questionnaire) will be collected while on study treatment every 8 weeks through the first year, then every 12 weeks thereafter. If the patient discontinues study treatment, assessment will occur at that time and then 8 weeks (± 2 weeks) later, regardless of status of subsequent treatment. PROs should be administered before conducting any other procedures.

Cycle ¹	Screening	1				2	Subsequent Cycles ²	Study Treatment Discontinuation (visit within 7 days of last dose)	Post Treatment Assessments
Day	-28 to -1	1 ³	8	15	21	1	Cycle n, Day 1		
Study treatment dispensed/collected		X				X	X	X ²⁵	
Anti-cancer therapies assessment									X ²⁶
Survival assessment ²⁷									X
Bone marrow aspirate and biopsy		X ²⁸							

²⁵ No new capsules dispensed

²⁶ Every 90 days following study treatment discontinuation.

²⁷ In addition to survival, this assessment includes outcomes for subsequent anticancer therapies including any new malignancy information.

²⁸ For any suspected MDS/AML case reported while a patient is receiving treatment or being followed for post-treatment assessments, bone marrow aspirate and biopsy testing must be completed by a local hematologist. Testing completed as part of standard of care is sufficient as long as the methods are acceptable to the Sponsor's Medical Monitor. The study site must receive a copy of the hematologist's report of aspirate/biopsy findings (which must include a classification according to WHO criteria²⁰) and other sample testing results related to MDS/AML.

7.2. Procedures by Visit – Main Study

Visits should occur within ± 3 days of the scheduled visit. All times should be recorded using the 24-hour clock (eg, 23:20, not 11:20 PM).

7.2.1. Screening (Visit 1, Day -28 to Day -1)

At Screening, the following procedures/tests will be performed:

- Written informed consent
 - Based on local site requirements, the following ICF procedures are acceptable:
 - Two separate ICFs will be signed. An ICF for the Myriad Integrated BRACAnalysis[®] test will be signed before the blood sample is drawn, prior to randomization. A main ICF will be signed prior to any other screening procedures (Day -28 to Day -1).
 - A single study ICF will be signed before any study procedures
- Demographics
- Medical/surgical/medication history, including history of prior myelosuppression (thrombocytopenia, neutropenia, leukopenia, or anemia) within the 1 year prior to signing the main ICF
- Cancer history
- Blood sample for centralized gBRCA mutation testing sent to Myriad for immediate Integrated BRACAnalysis[®] testing. (Testing must be completed prior to randomization. The sample may be submitted at any time prior to the screening period if it appears that the patient is likely to meet other eligibility requirements). DNA from this sample will be stored and may be used at a later time for biomarker testing including potential to bridging to candidate companion diagnostic assays
- Ascertainment of any prior gBRCA mutation test results, if performed
- Serum pregnancy test (within 72 hours prior to first dose) (for females of child-bearing potential only)
- Randomization (within 72 hours prior to first dose)
- Physical examination
- Vital signs (BP, pulse, temperature) and weight
- Height
- ECOG performance status
- CBC
- Coagulation
- Serum chemistry
- Serum CA-125

- Urinalysis (parameters must include: specific gravity, leukocyte esterase, nitrite, blood, protein, glucose, ketones, urobilinogen and bilirubin)
- 12-lead ECG
- Formalin fixed paraffin-embedded tumor sections of 100 micron thickness (≥ 80 micron minimum) or unsectioned paraffin block
- RECIST v.1.1 tumor assessment of the abdomen/pelvis and other areas (if clinically indicated)
- Chest CT/MRI if not done as part of RECIST tumor assessment. (If the chest CT/MRI is clear at screening, repeat chest imaging is not required in the absence of lesions to be followed or in the absence of clinical indication requiring follow-up)
- PROs (FOSI, EQ-5D-5L, neuropathy questionnaire).
 - Note that PROs should be administered before conducting any other procedures.
- AE monitoring
- Concomitant medications

7.2.2. Cycle 1, Day 1

- Physical examination
- Vital signs (BP, pulse, temperature, and weight)
- ECOG performance status
- AEs
- Concomitant medications
- CBC (repeat testing not required if Screening testing was done within 72 hours of Day 1)
- Coagulation (repeat testing not required if Screening testing was done within 72 hours of Day 1)
- Serum chemistry (repeat testing not required if Screening testing was done within 72 hours of Day 1)
- Serum CA-125
- 12-lead ECG predose and 2 hours postdose (prior to blood draws for PK)
- Blood sample for PK determination (predose within 30 minutes and 2 hours $\pm$ 15 minutes postdose); blood draws for PK are to be done after ECG monitoring and the exact time of the blood draw is to be recorded
- Blood sample for retrospective analysis of typing markers of DNA repair (eg, BRCA)
- For subset of patients (select sites), triplicate ECGs on Day 1 only at baseline (predose) and 1, 1.5, 2, 3, 4, 6 and 8 hours postdose

- For subset of patients (select sites) undergoing intensive PK on Day 1, blood sample at baseline (within 30 minutes predose) and 1, 1.5, 2, 3, 4, 6 and 8 hours ($\pm$ 2 minutes at all time points) postdose.
- Study treatment capsules dispensed; first dose administered at the site

7.2.3. Cycle 1, Day 8

- CBC

7.2.4. Cycle 1, Day 15

- Physical examination
- Vital signs (BP, pulse, temperature) and weight
- AEs
- Concomitant medications
- CBC
- Coagulation
- Serum chemistry

7.2.5. Cycle 1, Day 21

- CBC

7.2.6. Cycle 2, Day 1

- Physical examination
- AEs
- Vital signs (BP, pulse, temperature) and weight
- ECOG performance status
- Concomitant medications
- CBC
- Coagulation
- Serum chemistry
- Serum CA-125
- 12-lead ECG predose and 2 hours postdose (prior to blood draws for PK)
- Blood sample for PK determination (predose within 30 minutes and 2 hours $\pm$ 15 minutes postdose); blood draws for PK are to be done after ECG monitoring and the exact time of the blood draw is to be recorded.
- Study treatment capsules dispensed/collected, as appropriate

7.2.7. Day 1, Subsequent Cycles

- Physical examination
- AEs
- Vital Signs (BP, pulse, temperature) and weight
- ECOG performance status
- Concomitant medications
- CBC
- Coagulation
- Serum chemistry
- Serum pregnancy test at 3 month intervals from screening (Cycles 4, 7, etc)
- Serum CA-125
- Blood sample for PK determination predose only (within 30 minutes) on Cycle 4/Day 1 and Cycle 8/Day 1 only; the exact time of the PK blood draw will be recorded
- RECIST (v.1.1) tumor assessment of the abdomen/pelvis and other areas (if clinically indicated) after every 2 cycles through Cycle 14 and then after every 3 cycles
- PROs (FOSI, EQ-5D-5L, neuropathy questionnaire) every 2 cycles through Cycle 14, then after every 3 cycles.
 - Note that PROs should be administered before conducting any other procedures
- Study treatment capsules dispensed/collected, as appropriate

7.2.8. Study Treatment Discontinuation (within 7 days of last dose)

- Physical examination
- Vital signs (BP, pulse, temperature) and weight
- ECOG performance status
- AEs (SAEs recorded 30 days post study treatment discontinuation)
- Concomitant medications
- CBC
- Coagulation
- Serum chemistry
- Serum CA-125
- 12-lead ECG
- RECIST (v.1.1) tumor assessment of the abdomen/pelvis and other areas (if clinically indicated) radiographic assessment will continue until progression in the study-specified schedule for patients terminating study for reasons other than disease progression

- PROs (FOSI, EQ-5D-5L, neuropathy questionnaire)
 - Note that PROs should be administered before conducting any other procedures.
- Study treatment capsules collected

7.2.9. Post Study Treatment Assessments (Study Visit Not Required)

- PROs (FOSI, EQ-5D-5L, neuropathy questionnaire) will be completed one additional time at 8 ($\pm$ 2 weeks) weeks following the study treatment discontinuation visit.
 - Note that PROs should be administered before conducting any other procedures.
- Assessment of next subsequent anticancer therapies will be assessed every 90 days ($\pm$ 28days) following study treatment discontinuation visit. The following will be collected using source documentation:
 - Next anticancer therapy (name and/or class)
 - Date of start of subsequent therapies
 - Dose limiting toxicities
 - Best response (CR, PR, SD, PD)
 - Date of progression
- Survival status will be assessed every 90 days ($\pm$ 28days) following study treatment discontinuation visit. New malignancy information will also be collected as part of this assessment.

7.2.10. Unscheduled Assessments

- For any suspected MDS/AML case reported while a patient is receiving treatment or being followed for post-treatment assessments, bone marrow aspirate and biopsy testing must be completed by a local hematologist. Testing completed as part of standard of care is sufficient as long as the methods are acceptable to the Sponsor's Medical Monitor. The study site must receive a copy of the hematologist's report of aspirate/biopsy findings (which must include a classification according to WHO criteria²⁰) and other sample testing results related to MDS/AML.